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PROVIDER EDITION - Focused Action Report (Sample)

Session-Ready Strategies for Riley T.

A clinician-facing report on a single presenting challenge - with a graded, session-by-session plan you can put into practice this week.

FOCUS AREA

Transition-driven shutdowns

PREPARED FOR (CLIENT)

Riley T., Age 9

SERVICE

**Pediatric Occupational
Therapy**

ENGAGEMENT

45 minutes, weekly

White-label sample. This is a demonstration report built on fictional sample data to show partner providers the format and depth they would receive with their own branding. Every recommendation is derived from the (sample) questionnaire answers shown in the appendix.

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Powered by Pieces of Perception - white-label sample for partner review

PROVIDER ENGAGEMENT SNAPSHOT

How this report was scoped to your service

These details are answered by the **provider** at intake and shape every recommendation that follows - so the plan fits the way you actually work, not a generic template.

Service type	Pediatric Occupational Therapy
Session format	1:1, in-clinic
Session length & cadence	45 minutes, weekly
Setting	Outpatient clinic (sensory gym + table-work room)
Engagement goal	Increase tolerance for non-preferred and novel table-top tasks and reduce shutdown during in-session transitions.

Clinical summary

Riley presents with transition-driven shutdowns: at the moment of moving onto a non-preferred or novel task, verbal output drops, the body stills, and engagement ceases. The pattern is best understood as an anxiety-driven regulation response to uncertainty rather than non-compliance, which is why predictability - not increased demand - is the lever. This report translates the family's questionnaire answers into a session-ready plan: establish a previewable session architecture, target the transition moment with signaling and bounded choice, then grade in novelty one small step at a time, with a clear de-escalation protocol for in-session shutdowns.

PRESENTING PROFILE

What is happening, clinically

Presenting pattern

Transition-driven shutdowns - Riley disengages, goes quiet, and becomes physically still when moved off a preferred activity onto a non-preferred or novel task.

The Clinical Picture

Onset & course Reported by family for ~2 years; intensified after a mid-year classroom change. Most pronounced when a transition is unsignaled or time-pressured.	Frequency Several times per session is expected early; family reports daily occurrence at home around task changes.
Observable signs Verbal output drops to near-zero. Body stills or curls inward. Averted gaze. May grip a preferred object. Not oppositional behavior - a regulation shutdown under perceived demand.	Clinical inference Best understood as an anxiety-driven protective response to uncertainty, not refusal. The shutdown buys predictability. Pushing harder extends it; restoring predictability shortens it.
Antecedents / triggers Unsignaled transitions; novel materials introduced without preview; perceived evaluation ('let's see if you can'); sensory load late in session.	Protective conditions Visual or verbal previewing; a first-then structure; familiar materials; a brief movement/proprioceptive input before the demand; a predictable session opening ritual.

CLINICIAN QUICK REFERENCE

The one-page read on Riley

Designed to sit beside you in session, or to hand to a covering clinician. Everything here is drawn from the questionnaire answers.

THE PATTERN IN ONE LINE	Riley shuts down at the moment of transition to a non-preferred or novel task. The shutdown is a regulation response to uncertainty, not non-compliance.
WHAT IT LOOKS LIKE IN SESSION	Sharp drop in verbal output, body stilling, averted gaze, gripping a preferred object. Onset is the transition itself, not the task.
WHAT REDUCES IT	Preview the transition (visual + verbal first-then). Offer a bounded choice. Insert proprioceptive input before the demand. Keep the session opening identical each week.
WHAT EXTENDS IT	Repeating or escalating the demand, adding time pressure, narrating your own frustration, or removing the preferred item abruptly.
EARLY-WARNING SIGNS	Slower responses, shorter answers, increased object-focus, a glance toward the door. Intervene here - before the full shutdown - for the best result.
RE-ENTRY AFTER A SHUTDOWN	Lower demand to near-zero, restore predictability, offer regulating input, then re-approach the SAME task in a smaller step. Do not debrief in the moment.
SESSION-END SUCCESS LOOKS LIKE	Riley tolerated one previewed transition with a bounded choice and re-engaged within ~2 minutes of a wobble - without a full shutdown.

IN-SESSION LANGUAGE

What to say at the transition

The wording that regulates versus the wording that extends the shutdown - matched to this client's triggers.

SAY THIS	INSTEAD OF
"First the puzzle, then we choose the next thing together."	"Okay, all done - new activity now." (unsigned switch)
"In two minutes we'll move. I'll show you what's next."	"We need to hurry, come on." (time pressure)
"You can start with the blue one or the green one."	"Let's see if you can do this one." (perceived evaluation)
"Looks like a big feeling showed up. I'll wait with you."	"You're okay, it's not a big deal." (dismisses the state)
"Want to push the wall / squeeze the ball first?"	Re-issuing the same instruction louder or faster
"We can make this one smaller. Just the first step."	Removing the preferred item to force the transition

GRADED SESSION PLAN

A six-session arc you can start this week

Predictability first, then signaling and choice at the transition, then graded novelty. Pace to the client - advance only when the prior phase is stable.

Sessions 1-2: Establish predictability

The aim

Do not introduce new demands yet. Build a fixed, previewable session architecture so the transition itself stops being the threat.

In session:

- Open every session with the same 3-step ritual (greeting, a known proprioceptive warm-up, a visual schedule of the session).
- Use a visual first-then board for every transition, without exception, so the structure becomes predictable.
- Collect a simple baseline: count shutdowns per session and time-to-re-engagement. You are looking for a trend, not a number.
- Keep materials familiar this phase. Predictability before challenge.

Watch for:

- Expect shutdowns to continue early - you are installing structure, not yet reducing the response.
- If Riley begins glancing at the visual schedule unprompted, that is an early sign the structure is landing.

Sessions 3-4: Signal and choice at the transition

The aim

Now target the transition moment itself with previewing and bounded choice, so Riley experiences transitions as predictable and partly self-directed.

In session:

- Preview each transition ~2 minutes out, with both a verbal and a visual cue. Never switch tasks cold.
- Offer a bounded choice at every transition (which material, which order, where to sit) to restore a sense of control.
- Insert a brief proprioceptive input (wall push, ball squeeze, heavy work) immediately BEFORE the non-preferred task, not after.
- Catch the early-warning window: intervene at the first slowed response, before the full shutdown.

Watch for:

- A bounded choice can feel like 'too much' at first - keep it to two options.
- Track: did time-to-re-engagement drop below ~2 minutes on any transition this phase?

Sessions 5-6: Tolerating novelty in small steps

The aim

Begin grading in non-preferred and novel tasks deliberately, one small step at a time, with the predictability scaffolds still in place.

In session:

- Introduce one novel material per session, previewed and broken into its smallest first step.
- Pair each new task with an established regulating input and an immediate, specific reinforcer.
- Use 'we can make this smaller' as the standing repair move when a wobble appears - shrink the step, don't drop the task.
- Keep the opening ritual and visual schedule fixed - the constants are what make the novelty tolerable.

Watch for:

- Some regression is normal when novelty increases - hold the scaffolds steady rather than adding more.
- Looking for: one previewed novel task attempted with re-engagement and no full shutdown.

IN-SESSION DE-ESCALATION

If a shutdown happens mid-session

A step-order protocol. The throughline: reduce demand, restore predictability, offer regulation, re-approach smaller.

Step 1 - Reduce demand to near-zero

Stop issuing the instruction. Restating it extends the shutdown. Signal with your body that the demand is paused.

Step 2 - Restore predictability

Quietly name what is happening and what comes next: "We're going to wait. Then we'll pick." Predictability is the regulator here.

Step 3 - Offer regulating input, do not require it

Present a known proprioceptive or deep-pressure option (wall push, weighted lap pad, ball squeeze). Offer; never force.

Step 4 - Re-approach in a smaller step

Once verbal output and posture return, re-enter the SAME task at its smallest first step with a bounded choice. Do not jump ahead.

If shutdown deepens or distress escalates

Lower all demand, reduce sensory load (lighting, noise, proximity), and shift to a regulating activity. End on a known, tolerable task. Debrief with the caregiver after session, not with the child in the moment.

PROGRESS INDICATORS

How you will know it is working

Concrete, observable markers to track across the engagement. Re-rate against the early baseline.

- ✓ Shutdowns per session drop 50%+ from the Sessions 1-2 baseline.
- ✓ Time-to-re-engagement after a wobble falls below ~2 minutes.
- ✓ Riley tolerates a previewed transition with a bounded choice without a full shutdown.
- ✓ Riley orients to the visual schedule or first-then board unprompted.
- ✓ At least one novel task per session is attempted at its first step.
- ✓ Early-warning signs (slowed responses) are catchable before a full shutdown on most transitions.
- ✓ Caregiver reports the same previewing strategy reducing shutdowns at home (generalization).

SOURCE DATA

The questionnaire answers behind this report

Every recommendation in this report traces to one of these answers, reframed through a clinical lens. In production these come from the family's responses plus the provider's intake (last four rows).

Q. Child's first name

A. Riley

Q. Last initial

A. T.

Q. Age

A. 9

Q. Pronouns

A. they/them

Q. Diagnoses (sample)

A. Autism / ASD Level 1; Generalized Anxiety

Q. Communication style

A. Verbal in comfortable settings; verbal output drops sharply under perceived demand or uncertainty.

Q. The specific challenge being addressed

A. Shuts down (goes quiet, body stills, disengages) when moved off a preferred activity onto a non-preferred or novel one.

Q. When it started

A. About 2 years ago; intensified after a mid-year classroom change.

Q. How often

A. Daily at home around task changes; expected several times per early session.

Q. What it looks like

A. Near-zero verbal output, body stilling or curling inward, averted gaze, gripping a preferred object.

Q. Caregiver's read on the feeling underneath

A. Overwhelmed by not knowing what's next; the stillness is protective, not defiant.

Q. What seems to trigger it

A. Unsignaled transitions, time pressure, novel materials with no preview, feeling evaluated.

Q. What helps / when it does NOT happen

A. Previewing what's next, first-then structure, familiar materials, a movement break before the task, a predictable routine.

Q. Phrases that work at home

A. "First ____, then ____." "In two minutes we'll switch - here's what's next."

Q. Phrases to avoid

A. "Hurry up." "You're okay, it's not a big deal." Repeating the instruction louder.

Q. Provider service type (provider-answered)

A. Pediatric Occupational Therapy

Q. Session format & length (provider-answered)

A. 1:1 in-clinic, 45 min weekly

Q. Setting (provider-answered)

A. Outpatient clinic - sensory gym + table room

Q. Engagement goal (provider-answered)

A. Increase tolerance for non-preferred / novel tasks; reduce transition shutdowns.

IMPORTANT

Please read before using this report

Sample / demonstration only

This is a white-label demonstration built on fictional data to show the report format. It does not describe a real child.

Not medical advice or diagnosis

Reports of this type are an informational resource, not a clinical assessment, diagnosis, or treatment plan, and do not replace the professional judgment of a licensed provider.

Provider judgment governs

A licensed provider should adapt every recommendation to the individual client and their own scope of practice. Strategies that help one child may not suit another.

White-label engine

When licensed, the report carries the provider's branding. The underlying report engine is provided by Pieces of Perception.